Cetuximab + cisplatin + fluorouracil

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · cetuximab / platinum / fluorouracil

Active medicines: Cetuximab + Cisplatin + Fluorouracil

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2 and 3	Once daily on the stated days NCIS reference; review interactions and the actual multi-day infusion plan.
Ondansetron	16 mg orally Day 1	Before chemotherapy Clinician may split to 8 mg twice daily or reduce to 8 mg once daily. Review QT risk and electrolytes.
Dexamethasone · reconcile total steroid prescription	NCIS reference: 12 mg orally Day 1; 8 mg orally Days 2–4	As prescribed Cetuximab also requires a corticosteroid before infusion. One agreed total dose/route must replace duplicate prescriptions.
Olanzapine	5 mg orally once daily Days 1–4	As prescribed General high-risk NCIS row. Review sedation and metabolic/falls risk; metoclopramide combination increases extrapyramidal risk.
Metoclopramide · duration to be specified	10 mg orally three times daily in the NCIS subsequent-days row	Exact duration and indication must be prescribed The high-risk row does not state an end day or PRN qualifier. Check renal/neurological suitability; avoid unreviewed stacking with other dopamine antagonists.
Cetuximab antihistamine	Prescriber/pharmacy to specify agent, dose and route	At least one hour before every cetuximab dose The 00417/00418 source specifies the class and timing, not a dose. Confirm the total steroid alongside the antiemetic dexamethasone.
Cisplatin hydration and electrolytes	Individual pre- and post-hydration prescription required	Before and after Day 1 cisplatin Source includes suggested saline/magnesium guidance; individualise fluids and potassium.

Additional phase-specific support

- Give an antihistamine and a corticosteroid at least one hour before cetuximab; continue this premedication at subsequent doses. Reconcile steroids with any antiemetic prescription to avoid duplication.
- Ask specifically about earlier antibody reactions, red-meat allergy, tick bites and any cetuximab IgE result. Premedication does not eliminate severe or delayed reactions.
- Use a local EGFR-inhibitor skin plan, moisturiser and sun protection; assess painful, infected or intolerable rash and nail changes promptly.
- NCCP allows observation to cease after two reaction-free cetuximab doses, whereas current Erbitux SmPC section 4.2 requires monitoring during and for at least one hour after each infusion. Agree and document the governed observation plan; do not automatically remove observation.
- Prescribe regimen-specific hydration and electrolyte replacement with fluid-balance review. The appropriate volume depends on kidney and cardiac status; a generic printed plan is not an intravenous-fluid prescription.
- Review hearing, tinnitus and peripheral nerve symptoms before each cycle; damage can persist. Review nephrotoxic, ototoxic and other interacting medicines.
- Record the dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) result before the first fluorouracil exposure unless previously tested. Complete deficiency precludes fluorouracil; partial deficiency needs an individual starting dose and careful subsequent review.
- Normal DPD testing cannot exclude all severe fluorouracil toxicity. Explain unexpectedly early or severe diarrhoea, mucositis, neurological symptoms and chest pain.
- Write the actual infusion option, access arrangement, bag or pump changes and disconnect time. Do not equate four treatment days with a single universal pump duration.
- Provide an individually prescribed diarrhoea plan and hand/foot care advice; severe or persistent symptoms require assessment rather than repeated self-treatment.
- Arrange nutrition, weight and swallowing review early. Dietitian and speech and language therapy input should follow the person’s needs; swallowing safety cannot be inferred from a general leaflet.
- Use the agreed mouth-care and dental plan. Report painful swallowing, mouth ulceration, weight loss or inability to drink before the next appointment.

- Reconcile prescription, over-the-counter and complementary medicines. Discuss fertility, contraception and breastfeeding using the actual products and individual circumstances.
- Confirm the hospital's urgent contact and fever instructions. Infection, bleeding, severe diarrhoea, dehydration, chest pain or new breathlessness need prompt assessment, not the next routine review.
- Use this high-risk plan for the platinum treatment day. Do not repeat its full antiemetic course automatically at weekly cetuximab-only visits.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00417 v7 · pNCCP 00417: treatment and supportive-care sections; NCIS v8 pp4–5 where used · NCCP 00417 and current NCIS/local supportive-care guidance

Sources checked 2026-11-10 · SACTCheck v0.84.47 · Regimen-level drafts; prescriber and oncology-pharmacy verification required